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PNLE Nursing Starter Pack

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- ✓ 30 Questions Across All 5 PNLE Parts
- ✓ Fundamentals • MCN • CHN • MedSurg • Psych
- ✓ Full Rationales + Memory Hooks
- ✓ Built for 2026 Enhanced TOS (500-item exam)

For Aspiring Filipino Registered Nurses

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LisensyaPrep PNLE Nursing Starter Pack

2026 Edition • Free Community Edition • v1.0

📖 ABOUT THIS FREE REVIEWER 📖

This reviewer is our gift to the Filipino nursing community. With over 45,000 candidates taking the PNLE each year and pass rates fluctuating wildly (64% in May 2025, 90% in November 2025), proper preparation is the difference between joining the profession and waiting another 6 months.

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Welcome, Future Registered Nurse.

Salamat for downloading the LisensyaPrep PNLE Nursing Starter Pack! Inside are 30 carefully-crafted questions covering all 5 parts of the Philippine Nurses Licensure Examination.

Why this reviewer is different.

Most free PNLE reviewers online dump hundreds of questions with no explanation. You memorize answers, then fail to apply nursing judgment during the actual exam — where every question requires CRITICAL THINKING.

This reviewer is built differently. Every question includes:

- A detailed rationale explaining WHY the correct answer is correct
- Explanations for why each wrong choice is wrong
- Memory hooks for nursing concepts you must memorize
- Aligned with the 2026 Enhanced TOS (PRC Resolution No. 10, s.2025)

The Reality of the PNLE

The PNLE is a 500-item, 2-day exam. To pass, you need a 75% general average with NO subject below 60%. That last requirement — no subject below 60% — is where many examinees fail. You can't ignore your weak areas.

Major changes in 2026:

- Exam moved from May/November to FEBRUARY and AUGUST
- New Enhanced TOS uses Bloom's Taxonomy (6 cognitive levels)
- Higher emphasis on Application, Analysis, and Evaluation (not just recall)

Tara, simulan na natin!

2026 PNLE Coverage

Based on PRC Board of Nursing Enhanced TOS (Resolution No. 10, s.2025)

The PNLE consists of 500 multiple-choice items across 5 parts, administered over 2 days. Each part has 100 items and is timed for approximately 2 hours.

Part	Coverage	Items
Nursing Practice I	Fundamentals of Nursing + Professional Adjustments	100 items
Nursing Practice II	Community Health Nursing + Maternal/Child Care	100 items
Nursing Practice III	Medical-Surgical Nursing Part A	100 items
Nursing Practice IV	Medical-Surgical Nursing Part B + Psychiatric Nursing	100 items
Nursing Practice V	Medical-Surgical Nursing Part C	100 items

Foundational Sciences (Integrated Across All Parts)

- Anatomy and Physiology
- Microbiology and Parasitology
- Biochemistry
- Pharmacology
- Pathophysiology
- Nutrition and Diet Therapy

Passing Requirements

- General average: at least 75%
- No individual subject below 60% (CRITICAL — many fail here)
- Exam dates 2026: February 26-27 and August 29-30

This Starter Pack samples all 5 PNLE parts — 6 questions per part.

Nursing Practice I — Fundamentals

Foundation of Nursing + Professional Adjustments

Question 1. What is the FIRST step in the nursing process?

- A. Planning
- B. Assessment
- C. Implementation
- D. Evaluation

Correct Answer: B. Assessment

Rationale:

The NURSING PROCESS has 5 phases: ADPIE — ASSESSMENT (first), Diagnosis, Planning, Implementation, Evaluation. Assessment involves collecting subjective and objective data about the client. You cannot plan or intervene without first gathering data. This is the foundation of evidence-based, individualized nursing care.

Why the other choices are wrong:

- A: Planning comes AFTER assessment and diagnosis
- C: Implementation is the 4th step
- D: Evaluation is the LAST step

Memory hook: ADPIE: Assessment → Diagnosis → Planning → Implementation → Evaluation

Question 2. When taking a patient's vital signs, which is the NORMAL range for adult resting heart rate?

- A. 40-60 beats/min
- B. 60-100 beats/min
- C. 100-120 beats/min
- D. 120-140 beats/min

Correct Answer: B. 60-100 beats/min

Rationale:

NORMAL ADULT RESTING HEART RATE = 60-100 bpm. Below 60 = BRADYCARDIA (may be normal in athletes but watch for symptoms). Above 100 = TACHYCARDIA. Always assess context — fever, anxiety, exercise, medications affect HR. The apical pulse (at the heart) is auscultated for 1 FULL MINUTE if irregular.

Why the other choices are wrong:

- A: 40-60 is bradycardia
- C: 100-120 is tachycardia

- D: 120-140 is severe tachycardia

Memory hook: Normal adult HR: 60-100 (below=brady, above=tachy)

Question 3. A nurse is preparing to administer medication. According to the RIGHTS of Medication Administration, which is NOT one of the standard 'rights'?

- A. Right patient
- B. Right medication
- C. Right dose
- D. Right brand name

Correct Answer: D. Right brand name

Rationale:

The RIGHTS of Medication Administration are typically 6-10 depending on the source. The CORE 5 RIGHTS are: (1) Right Patient (verify with 2 identifiers), (2) Right Medication, (3) Right Dose, (4) Right Route, (5) Right Time. Extended rights include: Right Documentation, Right Reason, Right Response, Right to Refuse. 'Right brand name' is NOT a recognized right — generic vs. brand is a separate concern handled by physician orders.

Why the other choices are wrong:

- A, B, C are all CORE rights of medication administration

Memory hook: 5 Rights: PATIENT, MEDICATION, DOSE, ROUTE, TIME

Question 4. What is the BEST position for a client experiencing dyspnea?

- A. Supine
- B. Trendelenburg
- C. High Fowler's
- D. Lateral

Correct Answer: C. High Fowler's

Rationale:

HIGH FOWLER'S POSITION (head of bed elevated 60-90°) is best for dyspnea. It allows MAXIMUM LUNG EXPANSION by lowering the diaphragm, improving oxygenation. Used for: dyspnea, CHF, pneumonia, COPD exacerbations, feeding (to prevent aspiration). Other Fowler variations: Semi-Fowler's (30-45°) — for less severe respiratory distress.

Why the other choices are wrong:

- A: Supine COMPRESSES the diaphragm — WORSENS dyspnea
- B: Trendelenburg (head down) is for shock, not breathing
- D: Lateral is for unconscious patients (airway protection)

Memory hook: Dyspnea → High Fowler's (HOB 60-90°) for max lung expansion

Question 5. Florence Nightingale is considered the founder of:

- A. Public health nursing
- B. Modern nursing
- C. Nursing administration
- D. Holistic nursing

Correct Answer: B. Modern nursing

Rationale:

FLORENCE NIGHTINGALE (1820-1910) — 'The Lady with the Lamp.' Founded MODERN NURSING through her work in the Crimean War (1854) where she dramatically reduced soldier mortality through hygiene, ventilation, and nutrition reforms. Wrote 'Notes on Nursing' (1859) — first nursing textbook. Established first secular nursing school at St. Thomas Hospital (1860). Her birthday (May 12) is International Nurses Day.

Why the other choices are wrong:

- A: Lillian Wald founded public health nursing
- C: not Nightingale's primary contribution
- D: holistic nursing emerged later

Memory hook: Nightingale = Mother of Modern Nursing

Question 6. The nurse is documenting in the patient's chart. Which entry is MOST APPROPRIATE?

- A. Patient seems anxious
- B. Patient stated 'I feel scared,' RR 24, hands trembling
- C. Patient is being difficult
- D. I think the patient needs sedation

Correct Answer: B. Patient stated 'I feel scared,' RR 24, hands trembling

Rationale:

PROPER NURSING DOCUMENTATION must be: (1) OBJECTIVE (factual, observable) and SUBJECTIVE (patient's own words in quotes), (2) SPECIFIC and MEASURABLE, (3) FREE of personal opinions/judgments. Choice B includes the patient's exact words ('I feel scared'), objective measurement (RR 24), and observable finding (hands trembling).

Why the other choices are wrong:

- A: 'seems' is subjective interpretation
- C: 'difficult' is judgmental
- D: 'I think' is opinion, not assessment

Memory hook: Documentation: OBJECTIVE + SUBJECTIVE (in quotes) — no opinions

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Nursing Practice II — MCN & CHN

Maternal-Child Nursing + Community Health Nursing

Question 7. What is the EARLIEST sign of pregnancy?

- A. Quickening
- B. Amenorrhea
- C. Positive pregnancy test
- D. Fetal heart tones

Correct Answer: B. Amenorrhea

Rationale:

PRESUMPTIVE SIGNS of pregnancy (subjective, earliest): AMENORRHEA (missed period), nausea/vomiting, breast tenderness, fatigue, urinary frequency. PROBABLE SIGNS (objective, mid): positive pregnancy test (hCG), Chadwick's sign (bluish vagina), Goodell's sign (soft cervix), Hegar's sign (soft uterine isthmus). POSITIVE SIGNS (diagnostic): fetal heart tones, fetal movement felt by examiner, fetal visualization on ultrasound. Amenorrhea is typically NOTICED FIRST by the woman.

Why the other choices are wrong:

- A: Quickening (fetal movement felt by mother) occurs at 16-20 weeks
- C: Positive test is probable, occurs after amenorrhea
- D: Fetal heart tones are positive signs, audible by 10-12 weeks

Memory hook: *Pregnancy signs: Presumptive (subjective) → Probable (objective) → Positive (diagnostic)*

Question 8. What is the recommended position for a woman in labor with prolapsed umbilical cord?

- A. Supine
- B. Knee-chest or Trendelenburg

- C. Sitting
- D. High Fowler's

Correct Answer: B. Knee-chest or Trendelenburg

Rationale:

PROLAPSED UMBILICAL CORD is an EMERGENCY — cord compression cuts off fetal oxygen.

INTERVENTIONS: (1) Position mother in KNEE-CHEST or TRENDLENBURG (head down, hips up) to use GRAVITY to relieve cord compression, (2) Apply manual pressure on presenting part to lift it off the cord, (3) Administer oxygen, (4) Prepare for emergency C-section. Time is critical — fetal demise can occur within minutes.

Why the other choices are wrong:

- A: Supine WORSENS cord compression
- C: Sitting increases pressure on cord
- D: High Fowler's also increases cord compression

Memory hook: Cord prolapse = GRAVITY position (knee-chest or Trendelenburg) + STAT C-section

Question 9. A newborn's APGAR score at 1 minute is 7. The score includes which 5 components?

- A. Appearance, Pulse, Grimace, Activity, Respirations
- B. Anxiety, Pulse, Gait, Appetite, Reflexes
- C. Awareness, Posture, Gas exchange, Activity, Reflexes
- D. Apnea, Pressure, Glucose, Activity, Respirations

Correct Answer: A. Appearance, Pulse, Grimace, Activity, Respirations

Rationale:

APGAR SCORE (named after Dr. Virginia Apgar): assessed at 1 and 5 minutes after birth. Components scored 0, 1, or 2 each. APPEARANCE (color), PULSE (heart rate), GRIMACE (reflex irritability), ACTIVITY (muscle tone), RESPIRATIONS (breathing effort). Total: 0-10. 7-10 = good, 4-6 = moderate distress, 0-3 = severe distress requiring resuscitation.

Why the other choices are wrong:

- B, C, D: incorrect components

Memory hook: APGAR: Appearance, Pulse, Grimace, Activity, Respirations

Question 10. Which immunization is given at birth in the Philippines under the Expanded Program on Immunization (EPI)?

- A. MMR
- B. BCG and Hepatitis B
- C. Polio (OPV) only
- D. DPT only

Correct Answer: B. BCG and Hepatitis B

Rationale:

PHILIPPINE EPI SCHEDULE — at BIRTH: BCG (Bacillus Calmette-Guérin against tuberculosis) and Hepatitis B (HepB1). The first 6 weeks include: OPV/IPV, Pentavalent (DPT-HepB-Hib), Pneumococcal. MMR at 9 months. Influenza and HPV later. EPI prevents 7 major childhood diseases: TB, Diphtheria, Pertussis, Tetanus, Polio, Measles, Hepatitis B.

Why the other choices are wrong:

- A: MMR given at 9 months, not birth
- C: OPV starts at 6 weeks
- D: DPT starts at 6 weeks (as Pentavalent)

Memory hook: At birth: BCG + HepB1 (memorize this for community health)

Question 11. The primary level of disease prevention focuses on:

- A. Preventing disease before it occurs (immunization, health education)
- B. Early detection and treatment
- C. Rehabilitation
- D. Quarantine

Correct Answer: A. Preventing disease before it occurs (immunization, health education)

Rationale:

LEVELS OF PREVENTION (Leavell & Clark): PRIMARY — prevents disease BEFORE it occurs (immunizations, health education, nutrition counseling, safe sex education). SECONDARY — early detection and treatment (screenings: mammogram, BP check, blood sugar testing). TERTIARY — limits disability after disease (rehabilitation, physical therapy, support groups). Critical concept for community health nursing.

Why the other choices are wrong:

- B: Secondary prevention
- C: Tertiary prevention
- D: Specific intervention, not a level

Memory hook: Primary = Before disease. Secondary = Early detection. Tertiary = Rehab

Question 12. Which sign in a 6-month-old infant would indicate POSSIBLE child abuse?

- A. Soft anterior fontanel
- B. Multiple bruises in various stages of healing
- C. Loss of birth weight in first week
- D. Vernix caseosa at birth

Correct Answer: B. Multiple bruises in various stages of healing

Rationale:

MULTIPLE BRUISES in VARIOUS STAGES OF HEALING (yellow, green, purple, blue, red) on a child — especially a non-mobile infant — is a CLASSIC SIGN of CHILD ABUSE. A 6-month-old does not have the mobility to cause this through normal activity. Other red flags: injuries inconsistent with developmental stage, delayed seeking of care, inconsistent stories, fear of caregivers. Filipino nurses are MANDATED REPORTERS under RA 7610.

Why the other choices are wrong:

- A: Soft anterior fontanel is NORMAL at 6 months (closes 9-18 months)
- C: Loss of birth weight in first week is NORMAL (5-10% loss is expected)
- D: Vernix is normal newborn finding

Memory hook: Bruises in various stages = ABUSE red flag (especially in non-mobile child)

Nursing Practice III — Med-Surg A

Care of Clients with Physiologic Alterations (Part A)

Question 13. A patient with COPD is admitted with respiratory distress. What is the BEST oxygen flow rate to start with?

- A. 1-2 L/min via nasal cannula
- B. 6 L/min via nasal cannula
- C. 10 L/min via face mask
- D. 100% via non-rebreather

Correct Answer: A. 1-2 L/min via nasal cannula

Rationale:

In COPD, the body relies on the HYPOXIC DRIVE to breathe (low oxygen triggers breathing, since CO₂ levels are chronically elevated). HIGH-FLOW OXYGEN can REMOVE this drive and cause CO₂ RETENTION → respiratory arrest. Start LOW (1-2 L/min via NC) and TITRATE based on SpO₂ (target 88-92% for COPD, not 95%+). Monitor for confusion, lethargy (signs of CO₂ retention).

Why the other choices are wrong:

- B, C, D: too much O₂ can suppress respiratory drive in COPD

Memory hook: COPD = LOW oxygen flow (1-2 L). Target SpO₂ 88-92% (NOT 95%+)

Question 14. Which is the PRIORITY assessment for a patient with chest pain who may have an MI?

- A. Pain level
- B. Blood pressure
- C. Cardiac rhythm (ECG)
- D. Bowel sounds

Correct Answer: C. Cardiac rhythm (ECG)

Rationale:

In suspected MYOCARDIAL INFARCTION (heart attack), the PRIORITY is identifying life-threatening arrhythmias (V-fib, V-tach, asystole) that can cause sudden death. The ECG/EKG is the FIRST diagnostic tool — it shows ST elevation (STEMI) or depression (NSTEMI). REMEMBER 'MONA': Morphine, Oxygen, Nitroglycerin, Aspirin — but ECG comes FIRST. Time = muscle. Every minute of delay = more cardiac muscle death.

Why the other choices are wrong:

- A: Important but not priority over arrhythmia detection
- B: Important but ECG is more urgent

- D: Not relevant to immediate MI care

Memory hook: MI priority: ECG FIRST (catches lethal arrhythmias) → MONA

Question 15. What is the FIRST step in caring for a patient who has a seizure?

- A. Place tongue depressor in mouth
- B. Restrain the patient
- C. Protect from injury and maintain airway
- D. Administer diazepam IV

Correct Answer: C. Protect from injury and maintain airway

Rationale:

SEIZURE NURSING ACTIONS: (1) PROTECT FROM INJURY — clear surrounding area, place soft item under head, loosen restrictive clothing, (2) MAINTAIN AIRWAY — turn to side after seizure stops (lateral position to prevent aspiration), do NOT put anything in the mouth (can break teeth or block airway), (3) Time the seizure (>5 min = status epilepticus = emergency), (4) Observe and document type, duration, and post-ictal state.

Why the other choices are wrong:

- A: NEVER put anything in mouth (can damage teeth, obstruct airway, get bitten)
- B: NEVER restrain (can cause injury)
- D: Medications come AFTER safety is ensured

Memory hook: Seizure = SAFETY FIRST (no objects in mouth, don't restrain)

Question 16. A patient's blood glucose is 45 mg/dL. The patient is conscious. What is the FIRST intervention?

- A. Give IV dextrose
- B. Call physician
- C. Give 15g fast-acting carbohydrate (juice, glucose tabs)
- D. Give insulin

Correct Answer: C. Give 15g fast-acting carbohydrate (juice, glucose tabs)

Rationale:

HYPOGLYCEMIA (blood sugar <70 mg/dL): Use the RULE OF 15 — if patient is CONSCIOUS, give 15g fast-acting carb (4 oz juice, 4 glucose tablets, 1 tbsp honey, regular soda). Recheck blood sugar in 15 minutes. If still <70 → repeat. Once normal → give complex carb + protein (peanut butter sandwich, crackers and cheese) to maintain. IF UNCONSCIOUS → IV dextrose or glucagon (cannot give oral if unconscious — aspiration risk).

Why the other choices are wrong:

- A: Only if patient is UNCONSCIOUS
- B: Not first priority

- D: Insulin WORSENS hypoglycemia (lowers BG more)

Memory hook: Hypoglycemia conscious = Rule of 15 (15g carb, wait 15 min, recheck)

Question 17. Which lab value indicates DEHYDRATION?

- A. Low hematocrit
- B. High urine specific gravity (>1.030)
- C. Low BUN
- D. Low sodium

Correct Answer: B. High urine specific gravity (>1.030)

Rationale:

DEHYDRATION lab findings: ELEVATED — Hematocrit (hemoconcentration), BUN, sodium (hypernatremia), urine specific gravity (>1.030 = concentrated urine). DECREASED — Urine output (<30 mL/hr in adults). Other signs: dry mucous membranes, poor skin turgor, tachycardia, hypotension, sunken eyes, weight loss. Normal urine specific gravity: 1.005-1.030.

Why the other choices are wrong:

- A: HIGH hematocrit in dehydration (less plasma, more concentrated)
- C: HIGH BUN in dehydration
- D: HIGH sodium typically in dehydration

Memory hook: Dehydration: ↑ Hct, ↑ BUN, ↑ Na, ↑ urine SG, ↓ urine output

Question 18. A patient is to receive 1000 mL of D5W over 8 hours. What is the IV flow rate in mL/hour?

- A. 100 mL/hr
- B. 125 mL/hr
- C. 150 mL/hr
- D. 200 mL/hr

Correct Answer: B. 125 mL/hr

Rationale:

IV FLOW RATE CALCULATION: Total Volume ÷ Total Time (hours) = mL/hr. 1000 mL ÷ 8 hours = 125 mL/hr. If asked for drops per minute (gtts/min): mL/hr × drop factor ÷ 60 minutes. Common drop factors: 10, 15, 20 gtts/mL (macro), 60 gtts/mL (micro/pediatric).

Why the other choices are wrong:

- A: 100 mL/hr would deliver only 800 mL in 8 hrs
- C, D: too fast

Memory hook: IV rate (mL/hr) = Total volume ÷ Total hours

Nursing Practice IV — Med-Surg B & Psych

Med-Surg Part B + Psychiatric Nursing

Question 19. The therapeutic communication technique that encourages the patient to continue talking is:

- A. Giving advice
- B. Asking 'why' questions
- C. Active listening with open-ended questions
- D. Changing the subject

Correct Answer: C. Active listening with open-ended questions

Rationale:

THERAPEUTIC COMMUNICATION TECHNIQUES: Active listening, OPEN-ENDED QUESTIONS ('Tell me more about...'), reflection, silence, summarizing, validation, focusing. NON-THERAPEUTIC: Giving advice ('You should...'), false reassurance ('Don't worry'), 'why' questions (put patient on defensive), changing the subject, judgmental statements. Open-ended questions invite the patient to share more.

Why the other choices are wrong:

- A: Giving advice is non-therapeutic
- B: 'Why' is confrontational
- D: Changing subject dismisses patient

Memory hook: Therapeutic: Open-ended Qs. NOT therapeutic: 'Why?', advice, false reassurance

Question 20. A patient with schizophrenia tells the nurse, 'The TV is sending me messages.' This is an example of:

- A. Hallucination
- B. Delusion of reference
- C. Illusion
- D. Flight of ideas

Correct Answer: B. Delusion of reference

Rationale:

DELUSION OF REFERENCE — false belief that ordinary events have special personal meaning (TV/radio sending messages, others talking about them in code). Types of delusions: Persecutory (being followed/harmed), Grandeur (special powers/identity), Reference (ordinary events have personal meaning), Somatic (false beliefs about body). HALLUCINATIONS are false sensory perceptions (hearing voices). ILLUSIONS are misinterpretations of real stimuli.

Why the other choices are wrong:

- A: Hallucinations are sensory (hearing/seeing things not there)
- C: Illusion misinterprets a real stimulus
- D: Flight of ideas = rapid topic shifts

Memory hook: *Delusion = false BELIEF. Hallucination = false PERCEPTION (senses)*

Question 21. Which is a CONTRAINDICATION for thrombolytic therapy in stroke patients?

- A. Onset of symptoms 2 hours ago
- B. Active bleeding or recent surgery
- C. Hypertension
- D. Headache

Correct Answer: B. Active bleeding or recent surgery

Rationale:

THROMBOLYTIC THERAPY (tPA — tissue plasminogen activator) dissolves clots to restore blood flow. WINDOW: must be given within 3-4.5 hours of ischemic stroke onset. CONTRAINDICATIONS: ACTIVE BLEEDING, recent surgery (within 2 weeks), recent stroke or trauma, brain tumor, bleeding disorders, hemorrhagic stroke (CT scan must rule this out first — tPA in hemorrhagic stroke = fatal). Always confirm ISCHEMIC stroke before thrombolytics.

Why the other choices are wrong:

- A: 2 hours is WITHIN window (good candidate)
- C: HTN may be managed before tPA
- D: Headache is a symptom, not contraindication

Memory hook: *tPA contraindications: BLEEDING (active or recent surgery) = STOP*

Question 22. A patient on lithium for bipolar disorder has a level of 1.8 mEq/L. The nurse should:

- A. Continue the medication
- B. Encourage water intake
- C. Hold the dose and notify physician (toxic level)
- D. Increase the dose

Correct Answer: C. Hold the dose and notify physician (toxic level)

Rationale:

LITHIUM THERAPEUTIC LEVEL: 0.6-1.2 mEq/L (some sources 0.5-1.5). TOXIC LEVEL: >1.5 mEq/L. 1.8 mEq/L = TOXIC. Toxicity symptoms: COARSE TREMORS, ataxia, slurred speech, vomiting, confusion, seizures, coma. Sodium and dehydration ↑ lithium levels — encourage adequate sodium and fluid intake. NSAIDs and dehydration ↑ toxicity. Hold dose, notify doctor STAT, prepare for serum level recheck.

Why the other choices are wrong:

- A: Continuing increases toxicity
- B: Water intake doesn't fix immediate toxicity
- D: Increase WORSENS toxicity

Memory hook: *Lithium: Therapeutic 0.6-1.2, Toxic >1.5 → HOLD + NOTIFY*

Question 23. The PRIORITY nursing intervention for a patient experiencing acute mania is:

- A. Encourage participation in group therapy
- B. Provide a low-stimulus environment and safety
- C. Engage in lengthy discussion
- D. Set strict rules

Correct Answer: B. Provide a low-stimulus environment and safety

Rationale:

ACUTE MANIA: hyperactive, decreased need for sleep, racing thoughts, grandiose, impulsive, may have psychotic features. NURSING PRIORITIES: (1) SAFETY — protect from injury/exhaustion, (2) LOW-STIMULUS ENVIRONMENT — reduce stimulation that worsens mania, (3) Encourage adequate nutrition (may forget to eat), (4) Set CONSISTENT but not overly strict limits, (5) Administer mood stabilizers (lithium, valproate).

Why the other choices are wrong:

- A: Groups too stimulating during acute mania
- C: Lengthy talks too stimulating
- D: Strict rules can escalate behavior

Memory hook: *Acute mania = LOW STIMULUS + SAFETY first*

Question 24. Which assessment finding indicates a possible SUICIDE risk and requires IMMEDIATE intervention?

- A. Patient says 'I feel sad sometimes'
- B. Patient gives away personal belongings and says 'I won't need this anymore'
- C. Patient enjoys family visits
- D. Patient eats meals regularly

Correct Answer: B. Patient gives away personal belongings and says 'I won't need this anymore'

Rationale:

SUICIDE WARNING SIGNS (HIGH RISK): Giving away possessions, saying goodbye, sudden CALM after depression (decision made), specific plan with means available, previous attempts, hopelessness, substance use. NURSE ACTIONS: (1) Ask DIRECTLY 'Are you thinking of harming yourself?' (asking doesn't plant the idea — it opens dialogue), (2) Implement SUICIDE PRECAUTIONS (1:1 observation, remove dangerous items), (3) Stay with patient, (4) Document and notify team. NEVER LEAVE ALONE.

Why the other choices are wrong:

- A: Mild depression statement, not immediate risk
- C: Healthy social engagement
- D: Normal behavior

Memory hook: *Suicide red flags: giving away possessions + saying goodbye = IMMEDIATE*

Nursing Practice V — Med-Surg C

Care of Clients with Physiologic Alterations (Part C)

Question 25. A patient post-thyroidectomy develops carpopedal spasm and positive Chvostek's sign. This indicates:

- A. Hyperthyroidism
- B. Hypocalcemia (parathyroid damage)
- C. Hyperkalemia
- D. Respiratory distress

Correct Answer: B. Hypocalcemia (parathyroid damage)

Rationale:

POST-THYROIDECTOMY COMPLICATION: HYPOCALCEMIA from accidental PARATHYROID GLAND damage/removal. Parathyroid glands regulate calcium. SIGNS of HYPOCALCEMIA: CHVOSTEK'S SIGN (facial twitching when cheek tapped), TROUSSEAU'S SIGN (carpopedal spasm with BP cuff inflation), tetany, numbness/tingling around mouth, muscle cramps, laryngospasm (airway emergency!). Treatment: IV calcium gluconate. Always keep IV calcium at bedside for thyroidectomy patients.

Why the other choices are wrong:

- A: Hyperthyroidism is what surgery treats
- C: Hyperkalemia has different signs
- D: Could occur but specific signs point to hypocalcemia

Memory hook: Post-thyroidectomy + Chvostek/Trousseau = HYPOCALCEMIA (give Ca gluconate)

Question 26. What is the FIRST sign of increased intracranial pressure (ICP)?

- A. Cushing's triad
- B. Change in level of consciousness
- C. Pupil dilation
- D. Decerebrate posturing

Correct Answer: B. Change in level of consciousness

Rationale:

INCREASED ICP — EARLIEST SIGN is CHANGE IN LEVEL OF CONSCIOUSNESS (LOC) — confusion, restlessness, lethargy, irritability. LATER SIGNS: Pupil changes (sluggish, then unequal, then fixed/dilated), motor weakness, posturing (decorticate → decerebrate). LATE/OMINOUS: CUSHING'S TRIAD (↑ systolic BP with widening pulse pressure, ↓ HR, irregular respirations) — signals impending herniation. Normal ICP: 5-15 mmHg.

Why the other choices are wrong:

- A: Cushing's triad is a LATE sign
- C: Pupil changes occur AFTER LOC change
- D: Posturing is a LATE sign

Memory hook: ICP first sign: LOC change. Last sign: Cushing's triad (BP↑, HR↓, RR irregular)

Question 27. A patient is admitted with a hemoglobin of 6.5 g/dL. The PRIORITY intervention is:

- A. Bed rest
- B. Iron supplements
- C. Blood transfusion
- D. Oxygen therapy

Correct Answer: C. Blood transfusion

Rationale:

NORMAL HEMOGLOBIN: Males 13.5-17.5 g/dL, Females 12-15.5 g/dL. SEVERE ANEMIA (<7 g/dL) typically requires BLOOD TRANSFUSION (packed RBCs). Iron supplements take WEEKS to raise Hgb — not for acute severe anemia. PRE-TRANSFUSION: Verify with 2 nurses, check vital signs, ensure patent IV (18g preferred), only use 0.9% NaCl as compatible solution. MONITOR FIRST 15 MINUTES closely for transfusion reactions. Oxygen helps but doesn't treat the cause.

Why the other choices are wrong:

- A: Bed rest is supportive, not corrective
- B: Iron is slow-acting, not for acute
- D: O2 is supportive but doesn't replace RBCs

Memory hook: Severe anemia (Hgb <7) = TRANSFUSE PRBCs (with 0.9% NaCl)

Question 28. Which client should the nurse assess FIRST?

- A. A diabetic with blood sugar 180 mg/dL
- B. A post-op patient with stable vitals
- C. A patient with chest pain and shortness of breath
- D. A patient who wants a discharge prescription

Correct Answer: C. A patient with chest pain and shortness of breath

Rationale:

PRIORITIZATION uses ABC + Maslow + Acuity. AIRWAY → BREATHING → CIRCULATION are top priorities. CHEST PAIN + SOB = possible MI, PE, or other life-threatening emergency. ASSESS FIRST. After ruling out emergencies, address: unstable patients → stable patients → routine care. Use the 'who could DIE first' rule when prioritizing.

Why the other choices are wrong:

- A: Mildly elevated BG, not emergency
- B: Stable, can wait
- D: Non-urgent administrative task

Memory hook: Prioritize: ABC + 'who could die first' — chest pain/SOB always urgent

Question 29. A patient with renal failure has a potassium of 6.5 mEq/L. The most CRITICAL complication is:

- A. Muscle weakness
- B. Cardiac arrhythmia
- C. Constipation
- D. Headache

Correct Answer: B. Cardiac arrhythmia

Rationale:

NORMAL POTASSIUM: 3.5-5.0 mEq/L. HYPERKALEMIA (>5.0): >6.5 is EMERGENCY. Most dangerous complication: CARDIAC ARRHYTHMIAS (V-fib, asystole) — high K disrupts cardiac electrical conduction. ECG changes: PEAKED T WAVES (earliest), prolonged PR, widened QRS, sine wave (pre-arrest). TREATMENT: Calcium gluconate IV (stabilizes cardiac membrane), insulin + D50 (shifts K into cells), sodium polystyrene (Kayexalate — eliminates K), dialysis if severe. AVOID K-rich foods.

Why the other choices are wrong:

- A: Muscle weakness present but not most critical
- C: Not main concern
- D: Not main concern

Memory hook: Hyperkalemia → CARDIAC ARRHYTHMIAS (peaked T waves on ECG)

Question 30. A patient is taking warfarin (Coumadin). Which lab value should be monitored?

- A. aPTT
- B. INR/PT
- C. CBC only
- D. Liver enzymes only

Correct Answer: B. INR/PT

Rationale:

ANTICOAGULANT MONITORING: WARFARIN (Coumadin) — monitor PT (prothrombin time) and INR (international normalized ratio). Therapeutic INR usually 2-3 (or 2.5-3.5 for mechanical valves). Antidote: VITAMIN K. HEPARIN (UFH) — monitor aPTT (1.5-2.5x normal). Antidote: PROTAMINE SULFATE. LMWH (enoxaparin) — minimal monitoring needed. Patient teaching for warfarin: consistent VITAMIN K intake (leafy greens), avoid abrupt diet changes, watch for bleeding signs.

Why the other choices are wrong:

- A: aPTT monitors heparin, not warfarin
- C: CBC is supportive but not primary
- D: Not primary monitoring

Memory hook: Warfarin → INR/PT (antidote: Vit K). Heparin → aPTT (antidote: Protamine)

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